



|                   |                     |                         |                     |                            |                          |
|-------------------|---------------------|-------------------------|---------------------|----------------------------|--------------------------|
| <b>Name</b>       | Mr.SUKHWINDER SINGH | <b>Sample Received</b>  | 25-12-2024 03:45 PM | <b>Sample Collected At</b> | Cure N Care Nursing Home |
| <b>Age/Gender</b> | 68 Y / Male         | <b>Sample Processed</b> | 25-12-2024 03:46 PM | <b>Referred By</b>         | Dr Sukanya Gupta         |
| <b>UID</b>        | HLD2512202447870    | <b>Approved Date</b>    | 28-12-2024 03:17 PM |                            |                          |

## MICROBIOLOGY REPORT

### CULTURE AEROBIC PUS

|                                          |   |                         |   |  |  |
|------------------------------------------|---|-------------------------|---|--|--|
| Culture, Aerobic (PUS)                   |   |                         |   |  |  |
| Culture Result - <b>POSITIVE</b>         |   |                         |   |  |  |
| Organism - <b>Pseudomonas aeruginosa</b> |   |                         |   |  |  |
| Ampicillin                               | R | Imipenem                | S |  |  |
| Gentamicin                               | S | Ceftazidime             | S |  |  |
| Colistin                                 | S | Meropenem               | S |  |  |
| Tobramycin                               | S | Fosfomycin              | S |  |  |
| Cefuroxime                               | R | Nitrofurantoin          | R |  |  |
| Cefepime                                 | S | Piperacillin Tazobactam | S |  |  |
| Levofloxacin                             | S | Tigecycline             | S |  |  |
| Ciprofloxacin                            | S | Cefotaxime              | S |  |  |
| REMARKS : Kindly correlate clinically.   |   |                         |   |  |  |

The biological reference range pertains to the age and sex of the Patient.

**Note:** As the reports are computer generated, it must be treated as an original.

**Dr Lubna Samad**  
 Consultant Microbiologist  
 Reg.No. JK-3307

## A Unit Of Hexamed Healthcare Services LLP

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**CONDITIONS / PREREQUISITE OF LABORATORY REPORTING**

It is presumed that the specimen belongs to the patient as identified in the test requisition form. The report will carry the name and age provided at the time of registration. The patient/referring doctor is requested to immediately notify any demographic error on the report with respect to Name, Gender, Age, Investigation report heading, etc., as may be necessary.

The reported results are best evaluated by the referring doctor or the concerned medical specialist.

Please note that results of the test(s) may vary from laboratory to laboratory depending on the technology and methodology used. Furthermore, some parameters may vary from time to time even for the same patient. Sometimes test(s) may have to be repeated in the interest of quality assurance. In some cases, the test(s) may be referred to another competent laboratory for cross verification of the results.

In certain instances, a second sample may be required from the patient on account of an intermediate result, or pre-analytical/analytical reason.

If your report carries a 'PRELIMINARY' status, it means that results for one or more test(s) are awaited specifically in case of microbiology tests. 'FINAL' culture report may be issued only after 2 to 3 days from the date of receipt of the sample, to facilitate its proper identification and the subsequent drug test(s) for its susceptibility.

Some tests are referred to other laboratories to provide a wider test menu to the patient. The details of the laboratory where the sample was referred to, can be obtained from the Customer Services desk of the facility. In such cases, the test reports may be delayed, which will be marked as # on the information booklet or information chart.

In the case of alarming and unexpected test results, you are advised to contact the laboratory immediately for further discussion and action. Laboratory results are meant to be correlated with the patient's clinical history. In such circumstances, please call 1800-212-6547.

Neither Hexamed Diagnostics & Specialty Clinics nor its directors/employees/ representatives assume any liability or responsibility, for any loss or damage or expenditure that may be incurred by any person, including the patient, as a result of assuming the meaning or contents of this report without further validation. Reporting of tests will be as per the defined laboratory turnaround time (TAT) for each test. The same will be informed to the patient during registration and / or sample collection. Delay in issuing test reports may occur due to unavoidable/unforeseen circumstances like shortage or non-availability of the test kits or instrument failure etc.

Please note: Histopathological specimens are retained for 6 months only from the date of the sample collection and blocks and slides are retained for 5 years from the date of its reporting. A minimum of 48 hours prior notice is required for the issuing the slides and blocks for such tests.

Test results are not valid for Medico Legal purposes. To maintain confidentiality, certain reports may not be sent over an email at the discretion of the management.

Reports can be received either as a hard copy or on an email on your personal mail id. Please note: Only reports of patients with no dues are sent over e-mail/uploaded or dispatched.

Home sample collection facilities are provided with prior appointment. Request for the same is to be made on 1800-212-6547 or booked online at [www.hexamedhealthcare.com](http://www.hexamedhealthcare.com)

In case of any queries pertaining to your test results or to provide feedback/suggestions please call us on 1800-212-6547 or email us at [feedback@hexamedhealthcare.com](mailto:feedback@hexamedhealthcare.com)

Claims if any, are subject to the jurisdiction of the Courts of Law in the Union Territory of Jammu and Kashmir.

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